

TRANSFORMING HEALTHCARE

A Five-Pillar Framework with Vermont as the National Proving Ground

Bechir BenSaid

Fully Revised & Audited Source Text (Fall 2026 Edition)

Table of Contents (Volume I: Core Framework)

(Note: Table of Contents field codes will automatically update page numbers when opened in Microsoft Word or Google Docs.)

PREFACE

This book began with a room full of people who already knew something was wrong.

In September 2024, the Green Mountain Care Board (GMCB) of Vermont convened the public meeting — recounted in detail in the Introduction that follows — at which the consulting firm Oliver Wyman presented the findings of a year-long, \$1 million statewide hospital systems analysis. The people in that room had come because they suspected what was about to be confirmed, and because some part of them still hoped they were wrong.

They were not wrong. Nine of Vermont's fourteen hospitals were already operating at a loss in FY2023. Thirteen of fourteen would be in the red by 2028, absent structural intervention. The cumulative five-year system deficit ranged from \$700 million to \$2.4 billion. Premiums had risen 108% in six years while household income grew 22%. The working-age population that funds the cross-subsidy keeping rural hospitals alive was declining. The over-65 population was growing toward 30% of the state.

■ BEYOND VERMONT — ON A TIME DELAY

"On a time delay" is measurable, not rhetorical. In 2020, three states had more residents over 65 than under 18: Maine, Vermont, and Florida. By 2024 there were eleven — Delaware, Hawaii, Montana, New Hampshire, Oregon, Pennsylvania, Rhode Island, and West Virginia had joined them. Vermont now ranks fourth-oldest in the country by median age (42.8), behind Maine (45.1), New Hampshire (43.0), and West Virginia (42.9).

The financial consequence is arriving on the same delay. Nationally, 41% of rural hospitals operate in the red (Chartis Center for Rural Health) and roughly 768 are at risk of closing, 315 of them within three years (Center for Healthcare Quality and Payment Reform). Vermont's

nine-of-fourteen — 64% — is not a different problem from the national one. It is the national problem, further along.

This book is the analytical framework for that change. It is not a policy brief for or against any particular agenda. The five goals of Vermont’s Act 167 — reduce inefficiencies, lower costs, improve health outcomes, reduce health inequities, and increase access to essential services — are goals that virtually every person across the political spectrum would endorse. The means for achieving them are legitimately contested. This book engages that contestation honestly and presents both sides where the evidence genuinely runs in two directions.

The framework is organized around five pillars: Policy, Technology, Economics, Clinical, and Operations. It emerged from the conviction that transformation fails when it addresses any single pillar in isolation. Payment reform without clinical redesign produces financial savings that evaporate when care models do not change. Clinical quality improvement without payment reform produces better care that the financial system immediately discourages. Technology investment without operational readiness produces platforms that no one uses. Equity goals without structural investment produce aspirations without outcomes.

That last failure is why equity does not appear here as a sixth pillar. It is the **Equity Imperative** — a single justice test that each of the five pillars must pass on its own terms. The Introduction explains why that distinction is structural rather than a matter of emphasis, and Chapter 1 applies the test pillar by pillar.

The five pillars must move together — which means the people responsible for each pillar must understand all the others.

Vermont runs through this book as its primary teaching case. Vermont is in this book not because it is large or powerful, but because it is transparent. The combination of a small state’s intimacy, a strong public-sector analytical tradition, and an active regulatory body in the legislature, the GACB, and the Agency of Human Services (AHS) has produced a documentary record — the multiple legislative acts, the Oliver Wyman analysis, the GACB’s monthly hospital data, the AHS transformation reports — that gives this book a foundation most states cannot match. Vermont’s transformation is happening in public, with detailed public data, and the lessons transfer.

This book is also not a prediction. The transformations described here are underway but unfinished, and their outcome is genuinely uncertain — the Introduction’s “What This Book Cannot Do” section says plainly what this book does and does not claim to know. What this book provides is the analytical framework for understanding what is happening and why, and for contributing to better decisions as the story unfolds. The decisions, and their consequences, belong to the people who make them.

Finally, this book is not, on its own, the whole of the argument. This book is the intellectual foundation of the Health Transformation Review, an open-source integrated ecosystem with three parts designed to work as one:

- The book (**healthtransformationreview.org/book**) you are reading — the framework, the dependency logic, the Vermont evidence, and the national comparisons.
- The platform (**healthtransformationreview.org**) — five pillar hubs, a Research Lab of nearly forty interactive analytical tools, an AI Analyst grounded in this book and Vermont’s public-domain source documents, a fifty-state dashboard, and a daily intelligence feed. Every argument in these pages that can be modeled is modeled there.
- The Academy (**healthtransformationreview.org/academy**) — a structured executive-education layer of courses, tracks, and lessons that teach the concepts hands-on and link directly to the tools that apply them.

The three are deliberately cross-linked. Each chapter ends with two doorways: an “Explore on the Platform” guide that maps the chapter’s argument to the specific tools that let you manipulate it, and a “Go Deeper — Academy” guide pointing to the lessons that teach it hands-on. The platform’s tools carry “From the Book” callouts back to the chapters that explain them; and the AI Analyst can answer a question and route you to the right tool or lesson in the same breath. Everything in the ecosystem is, in effect, an interactive expression of this book’s framework. You can read the book alone and it stands on its own — but the framework is meant to be run, not just read, and the platform is where you run it.

■ EXPLORE ON THE PLATFORM

The full ecosystem is at healthtransformationreview.org — read the book online with audio narration at healthtransformationreview.org/book, open the Research Lab at healthtransformationreview.org/research-lab, ask the AI Analyst at healthtransformationreview.org/chat, or start a course in the Academy at healthtransformationreview.org/academy. The Introduction that follows explains how the three components fit together and which to start with for your role.

The reader profiles and recommended reading paths are in the Introduction that follows. This material rewards a reading strategy calibrated to your role and your decisions — and to the tools and courses that go with it — not a linear march through the whole book.

Fall 2026

INTRODUCTION

September 18, 2024

The room was full before the presentation started.

The Green Mountain Care Board of Vermont had convened a public meeting at which Oliver Wyman Healthcare and Life Sciences Group would present findings from a year-long, \$1 million statewide hospital systems analysis commissioned under Act 167 of 2022. The analysis had not been produced in isolation: over the preceding year, Oliver Wyman had run more than 230 meetings across all 14 of Vermont’s Hospital Service Areas, engaging over 3,100 participants from more than 100 organizations — hospital leaders, state legislators and agency officials, the

Vermont Hospital Association, health economists, community advocates, and clinicians. Many of the people who had taken part in that engagement now watched its conclusions arrive. They had come because they already suspected what was about to be confirmed, and because some part of them still hoped they were wrong.

They were not wrong.

Oliver Wyman's lead consultant opened with the financial trajectory. Nine of Vermont's fourteen hospitals were already reporting operating losses, with the worst reaching negative 8.9%. If current trends continued — the same trends that had been building for a decade, the trends that voluntary reform had been addressing for years without reversing — thirteen of the fourteen would be in operating losses by 2028. The cumulative five-year system deficit ranged from \$700 million in the optimistic scenario to \$2.4 billion in the realistic one.

The next slide showed premiums. The average silver exchange plan premium had risen from \$456 per month in 2018 to \$948 in 2024 — a 108% increase. Median household income had grown 22% over the same period. Out-of-pocket maximums had more than doubled. The University of Vermont Medical Center, with approximately 50% of the state's hospital market share, was charging commercial payers roughly 358% of Medicare rates — RAND's published figure for inpatient and outpatient services combined, drawn from 2018–2020 claims — making it, by RAND's own accounting, the most expensive hospital in Vermont. Statewide, Vermont hospitals charge commercial payers 250–300% of Medicare on average, and the Green Mountain Care Board's own later analysis found outpatient imaging at some Vermont hospitals reaching 715–944% of Medicare. Those charges flowed directly into the premiums that Vermont families were paying, with no regulatory brakes.

The final slide showed the demographics. Vermont's working-age population was projected to decline 13% by 2040 — from 367,000 to approximately 318,000 — while the over-65 population rose from 21.7% toward 30% of the state's total. This shift would progressively move patients from commercial insurance, which pays above cost and funds the cross-subsidy that sustains hospitals, into Medicare and Medicaid, both of which reimburse below cost. The slide did not say the words “system collapse.” It did not need to.

Vermont Is Not an Outlier. It Is a Preview.

No one in that room disputed the numbers. No one argued that the trend would reverse itself. The question that followed — the question this book is an attempt to answer — was whether structural change was possible. Not aspirational change. Not incremental improvement. But actual change to the fundamental incentives, relationships, and capabilities of the system. Whether a small rural state could force a healthcare transformation that the national system, with all its resources and political energy, had been deferring for decades.

Vermont's answer, enacted in Acts 167, 51, and 68 and partially funded by the federal government, is the case study used in this book. Whether it constitutes successful transformation will be determined over the next five to ten years. What is already clear is that Vermont has committed to structural change with more specificity, more statutory force, and more capital investment than any comparably sized system in recent American health policy history.

This book is written in real time (anchored in Fall 2026), closely watching that transformation. Not as a retrospective account of a completed success, but as an analytical framework for a transformation in progress — with all the uncertainty, potentially incomplete public information, and instructiveness that implies.

What Transformation Actually Means

Few words in healthcare are used more often than “reform” or “transformation,” and the frequency has cost them precision. Both get applied to routine process improvements and to genuine structural change alike, and to aspiration as readily as to commitment — which makes it worth stating, at the outset, exactly what this book means by transformation.

It uses “transformation” in a precise sense: transformation is structural change that alters the fundamental incentives, relationships, and capabilities of a system, producing different behaviors and different outcomes rather than incremental improvements within the existing structure.

By this definition, most of what has been called healthcare transformation over the past fifteen years has not been transformational. Voluntary accountable care organization programs that high-cost providers can decline to join are not transformational. Shared savings models that reward hospitals for reductions in utilization they would have achieved anyway are not transformation. Electronic health record adoption that digitizes paper processes without changing care delivery is not transformation. Quality measurement initiatives that produce reports without affecting payment are not transformation.

Transformation, in the sense this book uses the term, looks like what Vermont is attempting: mandatory reference-based pricing that eliminates the ability of dominant hospitals to charge commercial payers three to four times Medicare rates; statutory global hospital budgets (Act 68) that change every hospital’s financial incentive from maximizing volume to managing population health; a Statewide Health Care Delivery Strategic Plan that commits Vermont’s Agency of Human Services to specific, measurable outcomes by specific dates; and capital investment through the Rural Health Transformation Program (\$195M/year) that builds the community infrastructure that payment reform requires to produce population health improvement rather than just cost reduction.

How to Read This Book If Vermont Isn’t Your Context

This book uses Vermont as its primary teaching case for the reasons explained above: transparency, documentation, and an early position on a curve every state is on. But the subject of this book is the five-pillar framework — Policy, Technology, Economics, Clinical, and Operations — and the dependency logic that links them. Vermont is the evidence, not the scope. If Vermont is not your context, here is how to get the most out of the chapters that follow.

Translate the institutions, not just the policies. Every named Vermont statute or institution in this book is a specific instance of a general category that exists, in some form, in every state. The Green Mountain Care Board is Vermont’s version of a hospital rate-setting or cost-growth-benchmark authority — Massachusetts, Rhode Island, and several other states have analogous bodies under different names. Acts 167 and 68 are Vermont’s versions of a diagnostic mandate and an operational mandate — the categories, not the bill numbers, are what transfer.

AHS is Vermont’s version of a state health and human services agency acting as a system operator. As you read, ask: what is the equivalent institution, statute, or agreement in my state — even if it is smaller, newer, less formal, or does not yet exist?

Look for the framework statements, not only the Vermont data. Each chapter makes a claim about how the pillars depend on one another that does not require Vermont to be true — for example, that technology investment must precede payment reform because payment reform without data infrastructure cannot be measured or trusted. Vermont’s experience is the proof, presented in detail because detail is what makes an argument verifiable rather than asserted. But the claim itself is the point.

Use the “Implications for You” sections as your entry point. Most chapters close by addressing different reader roles directly — hospital executives, payers, state officials, clinicians. These sections are written to apply regardless of any state context, and are often the fastest way to find the actionable content inside a Vermont-heavy chapter. If you read nothing else in a chapter, read that section, then go back for the supporting detail you need.

The Strongest Case Against This Book’s Argument

This book argues that structural, mandatory reform can succeed where voluntary reform has failed. That is a contested claim, and intellectual honesty requires engaging the strongest version of the opposition before asking the reader to proceed on that premise.

1. The Price Regulation Argument

Economists and hospital advocates argue that mandatory reference-based pricing — tying hospital payment to a percentage of Medicare rates — will reduce access rather than improve affordability. If hospitals cannot cover their costs at regulated prices, they will close service lines, exit markets, or reduce Medicaid volumes. Vermont, by this reading, is not pioneering a solution. It is forcing a reckoning that will harm the rural communities it claims to protect.

This is a serious argument. Maryland’s experience with hospital global budgets is the primary counterevidence: Maryland has operated all-payer rate regulation since 1977 and maintains rural hospital access comparable to or better than non-regulated states, while holding per-capita cost growth below the national average. But Maryland’s model is decades old, built during a different regulatory and demographic environment. Vermont’s experiment is newer, its hospitals more financially fragile, and its rural population more dispersed. The access-reduction risk is real and this book does not dismiss it. Chapter 14 (Political Sustainability) analyzes it directly as the primary failure mode to monitor.

2. The Administrative Burden and Provider Resistance Argument

A second critique argues that heavy state regulation and rate-setting impose excessive administrative friction, driving clinicians away and creating adversarial relationships between state regulators and hospital systems.

The counterevidence is OneCare itself: a decade of voluntary reform produced modest results and left the financial crisis unaddressed. The real choice is between a process that produces structural change — contentiously — and one that produces durable accommodation of the status quo, smoothly. Vermont’s legislature chose structural change.

3. The Federal Model Reliability and Durability Argument (AHEAD Model Resolution)

CMMI has operated innovation models for fifteen years. Most have not produced the savings they projected. AHEAD's predecessor — the Vermont All-Payer ACO Model — produced modest results over nine years despite significant federal and state investment. Why will this time be any different?

This is perhaps the strongest challenge in the book, and the honest answer begins by conceding the premise: CMMI models have generally underperformed their projections, and several have been redesigned or discontinued mid-course. What is different about Vermont's approach is the mandatory statutory architecture its predecessor lacked. Acts 167 and 68 require hospitals to participate in global budgets and reference-based pricing regardless of what happens federally. Vermont's transformation does not depend on AHEAD; AHEAD was to amplify it.

This ceased to be hypothetical in July 2026, when Vermont formally withdrew from CMMI AHEAD after a CMS renegotiation that cut Vermont's expected EAST Fund from roughly \$138 million to a cap near \$10 million. The mandatory state statutory framework under Acts 167 and 68 remained fully in force, and the reform continued without missing a step. The skeptic's question was answered by events — Vermont did not need AHEAD to survive the loss of AHEAD. (Note that CMMI's AHEAD model continues as an active total-cost-of-care framework in other participating states, such as Maryland, Connecticut, Hawaii, Rhode Island, and New York; Vermont's withdrawal reflects specific federal renegotiation terms rather than an invalidation of global budget principles.) Chapter 13 (The Future of Healthcare Transformation) returns to this directly.

4. The Oliver Wyman Independence Question

This book relies heavily on Oliver Wyman's Act 167 analysis. Oliver Wyman was paid \$1 million by the Vermont state government to produce a report that would support the case for reform. That is not independence. Readers should note that the Oliver Wyman financial projections — hospital deficit trajectories, administrative cost benchmarks, workforce shortage estimates — cannot all be independently verified from public sources, and the firm had a client relationship that may have shaped how it framed its conclusions.

This book treats Oliver Wyman's quantitative findings as estimates, not certainties, and notes throughout where specific figures cannot be independently confirmed. The structural argument — that Vermont's system faces a genuine financial crisis requiring structural response — is supported by public GMCB data, independent KFF analysis, and GMCB's own enforcement actions that do not depend on Oliver Wyman's projections.

5. The Vermont-Specificity Argument

The most common version of this objection does not concern any single policy. It is a reaction to the book as a whole: this is a book about one small, unusually homogeneous, unusually transparent state with an unusual regulatory body — does any of it actually apply to a large, diverse, politically fragmented state with a dozen competing health systems and no Green Mountain Care Board?

This objection deserves to be taken seriously rather than answered with a slogan, because the honest answer has two parts. The first part is that the five-pillar framework itself — the

dependency logic that determines sequence, the argument that payment reform without clinical redesign evaporates, that technology investment without operational readiness produces unused platforms, that equity gaps require targeted rather than averaged interventions — was not derived from anything unique to Vermont. It is derived from how health systems, payment, technology, and care delivery relate to one another as a matter of structure. A large, fragmented state experiences it in a more fragmented way, but the logic is identical. The second part is the limitation: this book cannot provide the equivalent depth for every state's institutions. What it provides is a diagnostic playbook.

6. The Transition-Window Argument

Reference-based pricing and global hospital budgets shift a hospital from a system where it can grow revenue by growing volume to one where revenue is fixed or capped. For hospitals already operating below break-even, the most dangerous financial period may not be the destination (FY2030 statewide global budgets) but the transition itself: the years in which RBP is phasing in, global budgets are not yet operational statewide, and underlying losses remain unaddressed. Chapter 6 (the Economics Pillar) addresses this transition window directly.

7. The Enforcement Question

A mandatory regime is only as real as its enforcement. What actually happens to a hospital that does not comply — that refuses a global budget, or continues billing above the reference-based-pricing ceiling? Act 68 gives GMCB subpoena authority and data-sharing power with the Department of Financial Regulation. GMCB's FY23 enforcement actions against UVMHC (\$80.3M overage) and RRMC (\$11.1M overage) were the first such enforcement actions in Vermont's regulatory history, and UVMHC's subsequent court challenge to that enforcement was decided against UVMHC. That outcome is the strongest available evidence that "mandatory" in this book's framework describes a regime that has been tested against its most resourced potential opponent and held.

The Five-Pillar Framework and the Equity Imperative: An Integrated Architecture

The central analytical contribution of this book is the five-pillar framework for healthcare transformation: Policy, Technology, Economics, Clinical, and Operations. It is not a checklist of things that should be done, but an integrated architecture for understanding why transformation attempts underperform when any pillar is addressed in isolation.

Each pillar is defined by a single diagnostic question:

- Policy asks: Is it permissible?
- Technology asks: Is it possible?
- Economics asks: Is it sustainable?
- Clinical asks: Is it effective?
- Operations asks: Is it executable?

An honest "no" to any one of them puts the transformation at risk, regardless of how confidently the other four answer yes.

There is a sixth question, and it is deliberately not a sixth pillar: Is it just? This is the Equity Imperative — a cross-cutting test that each of the five pillars must pass on its own terms, rather than a separate workstream competing with them for budget, staff, and attention. The

distinction is structural, not rhetorical. A sixth pillar could be sequenced last, funded separately, or treated as an adjunct. An imperative has none of those escape hatches: each pillar's deliverable carries an equity condition it must satisfy.

The HTR Ecosystem: How the Book, the Platform, and the Academy Work Together

This book is one of three integrated components of the Health Transformation Review ecosystem. The framework covered in this book is not only an argument to be followed — it is a system to be operated, and the platform (healthtransformationreview.org) and Academy (healthtransformationreview.org/academy) are where you operate it.

Component	What It Is	How It Connects to the Book
The Book	Transforming Healthcare — framework, dependency logic, Vermont evidence, national comparisons. Read online at healthtransformationreview.org/book .	Intellectual foundation. Everything else is an interactive expression of it.
The Platform	Five pillar hubs, Research Lab with ~40 interactive tools, AI Analyst, 50-state dashboard, daily Wire feed at healthtransformationreview.org .	Every argument here that can be modeled is modeled there. Each chapter ends with an 'Explore on the Platform' guide.
The Academy	Structured executive-education layer of courses, tracks, and lessons. 'Five Pillars, One Imperative' companion course at healthtransformationreview.org/academy .	Each chapter ends with a 'Go Deeper — Academy' guide naming specific hands-on lessons.

How to Use This Book: Reader Profiles

This book has four primary reader profiles. Each has a recommended path across the book, platform, and Academy:

- **The Healthcare Policy Professional:** Focus on Introduction, Chapter 1, Chapter 2 (Policy), Chapter 16 (AHS Restructuring), and Chapter 13 (Forecast). Platform: HTR Simulator & Policy Simulator at healthtransformationreview.org/research-lab/policy-quality?tab=policy. Academy: Track 2 ('Policy — Establish the Mandate').
- **The Healthcare Executive or Administrator:** Focus on Chapters 6-7 (Economics), Chapters 8-9 (Clinical), Chapters 4-5 (Technology), and Chapter 11 (Operations). Platform: VBC Readiness Assessment & Hospital Financial Stress Test at healthtransformationreview.org/research-lab/knowledge-workspace?tab=scorecard. Academy: Track 4 ('Economics').

- The Vermont Practitioner: Focus on Appendix A (Portrait), Chapter 1, Chapter 2 (Acts 167 & 68), and Chapter 16 (Restructuring). Platform: Vermont Concept pages & Hospital Dashboard at healthtransformationreview.org/dashboard/vermont/hospitals. Academy: Vermont Medicaid Learning Center at healthtransformationreview.org/academy.
- The Student or Researcher: Read sequentially. Use Research Lab tools as a sandbox at healthtransformationreview.org/research-lab and Academy Track 1 at healthtransformationreview.org/academy/getting-started.

What This Book Cannot Do

This book cannot predict whether Vermont's transformation will succeed. The outcome depends on thousands of decisions that will be made by hospital executives, primary care physicians, community health workers, legislators, regulators, and patients over the next five to ten years.

This book cannot substitute for the operational expertise required to execute transformation. Implementing a global budget requires actuarial expertise, regulatory knowledge, and operational experience that a book cannot provide.

This book cannot resolve the political contests that surround healthcare transformation. What this book CAN do is supply the analytical architecture, evidence base, and conceptual vocabulary for making sound transformation decisions.

Chapter 1: The Five-Pillar Framework and the Execution Sequence

Healthcare transformation is a system problem: five pillars — Policy, Technology, Economics, Clinical, and Operations — connected by nine dependency relationships that determine whether reform succeeds or fails. This chapter opens with the failure that proves the point — OneCare Vermont's collapse — then defines the five pillars, maps their interdependencies, shows what cascades when any one is missing, and turns to the question those dependencies force: in what order must the pillars be built? Vermont provides both the proof of concept and the canonical failure case — OneCare Vermont's collapse is the most instructive sequencing autopsy in recent American health policy.

1.1 The Problem with Single-Pillar Thinking

Between 2011 and 2024, CMMI tested over 70 innovation models. Only 4 met the statutory requirements for nationwide expansion. The pattern of failure across CMMI models is instructive. Models that addressed only the Economics pillar — changing what Medicare paid without addressing how care was delivered or whether providers had the infrastructure to respond — generated financial pressure without behavioral change. Models that addressed only the Clinical pillar produced clinical improvements that the payment system then penalized by reducing volume-based revenue. Models that required Technology pillar investment found that providers lacked the operational capacity to use the tools effectively.

Every pillar you leave out becomes the constraint that defeats the ones you built.

The problem in health policy is treating a list of requirements as an independent checklist. A checklist implies items can be checked in any order. This is wrong. The five pillars are an interdependent system where order and timing matter as much as content.

1.2 The OneCare Failure: A Sequencing Autopsy

The clearest evidence for dependency logic is OneCare Vermont — Vermont’s primary ACO vehicle under a Total Cost of Care model from 2013 through its wind-down at the close of 2025. OneCare failed because of three compounding sequencing errors:

1.2.1 Sequencing Failure 1: Economics Without Policy Foundation

OneCare attempted to operate a population-level payment model without a policy foundation capable of enforcing full participation. Federal ERISA law (upheld in *Gobeille v. Liberty Mutual Insurance Co.*, 2016) preempts state authority over self-insured commercial employer plans. On January 1, 2023, Blue Cross Blue Shield of Vermont exercised its legal right to opt out, removing ~93,000 enrollees (one-third of OneCare’s population) overnight due to data governance concerns and unproven value.

Act 68’s mandatory hospital global budgets and reference-based pricing represent the direct statutory response to this failure: state statutory authority over hospital rate-setting remains absolute, closing the voluntary opt-out loop on the hospital side.

WORKED EXAMPLE — One Decision Through the Model (Act 68 Global Budgets)

Pillar	Verdict on Mandatory Hospital Global Budgets
Policy (permissible?)	Yes. Act 68 makes global budgets binding on every Vermont hospital — eliminating voluntary opt-out where state authority is absolute.
Technology (possible?)	Not yet. Only if VHCURES and AHS-GMCB analytics can set and monitor each hospital's budget; without real-time data, it is a number no one can manage.
Economics (sustainable?)	Yes, by design. Inverts incentives — every avoided admission improves margin instead of cutting revenue.
Clinical (effective?)	Only if care delivery follows. Margin is real only when Blueprint care teams actually prevent avoidable admissions.
Operations (executable?)	Not yet. Running budgets across all 14 hospitals requires project-management capacity Vermont is still building.

1.2.2 Sequencing Failure 2: Economics Without Technology — The 'Managing Blind' Failure Mode

OneCare assumed financial risk without integrating claims data (financial record) and clinical data (medical record) in near-real time. A state audit revealed the utilization signature of an unmanaged population: patients attributed to OneCare visited the ED 36% more frequently than national peer ACOs while recording 18% fewer primary care visits. Assuming downside risk without real-time analytics guarantees unexpected financial losses.

1.2.3 Sequencing Failure 3: The Cascade

Losing 1/3 of attributed lives degraded risk modeling; delayed analytics prevented proactive care intervention; inability to prove savings destroyed provider and payer confidence. The three failures compounded until OneCare wound down at the end of 2025.

Figure 1.1 — The OneCare Failure Cascade Summary

Sequencing Gap	Structural Error	Direct Consequence	Cascade Effect
Policy (Stage 1)	Payer Mandate Gap: Gobeille v. Liberty Mutual blocked state authority over self-insured plans.	BCBSVT exits (Jan 2023), removing ~93k enrollees (1/3 of total population).	Total cost of care cannot be managed or predicted for a fluctuating population.
Technology (Stage 2)	Data Substrate Gap: Risk assumed without integrating real-time clinical + claims feeds.	Managing Blind: Auditor-documented 36% higher ED visits & 18% lower primary care visits.	Proactive intervention impossible; acute events unprevented; losses accelerate.
Economics (Stage 3)	Real total-cost risk assumed on top of Policy & Tech gaps.	Conflicting conclusions on net savings vs. net cost across independent audits.	Contested results -> loss of stakeholder confidence -> model collapse at end of 2025.

1.3 The Five Pillars and the Equity Imperative

The five pillars are capability domains that answer five distinct diagnostic questions:

- **Policy:** The legislative and regulatory environment creating mandatory architecture (Acts 167, 51, 68). Diagnostic: Is it permissible?
- **Technology:** Data infrastructure, APCD, HIE, analytics, and clinical technology (VHCURES, VITL, CIN). Diagnostic: Is it possible?
- **Economics:** Payment and financial incentive architecture (RBP $\leq$ 200% Medicare, Global Budgets FY28). Diagnostic: Is it sustainable?
- **Clinical:** Care delivery models, workforce, and care coordination (Blueprint PCMH, CoCM, CCBHC). Diagnostic: Is it effective?
- **Operations:** Organizational capacity, PMO, administrative simplification, and execution (14-hospital plans, RHRC, HSA Coordinators). Diagnostic: Is it executable?
- **Equity Imperative:** The cross-cutting plumb line every pillar must pass. Diagnostic: Is it just?

Figure 1.2 — The Five Pillars: Diagnostic Questions and Structural Roles

Pillar	Diagnostic Question	Structural Role & Output When Functioning
Policy	Is it permissible?	The mandatory architecture. Produces statutory mandates that eliminate voluntary opt-outs and set non-deferrable timelines.
Technology	Is it possible?	The data substrate. Produces attribution lists, real-time TCOC measurement, HEDIS stratification, and AI workflows.
Economics	Is it sustainable?	The incentive architecture. Produces payment models where population health management is financially rational and cross-subsidies are restructured.
Clinical	Is it effective?	The mechanism of change. Produces primary care transformation, behavioral health integration, and care management that prevents acute hospitalizations.
Operations	Is it executable?	The execution layer. Translates mandates, payment models, and clinical programs into executed organizational reality.

1.4 The Architecture of Interdependency: The Dependency Matrix

The five pillars are connected by nine directed dependencies. Read each relationship as Row Pillar -> Column Pillar:

Figure 1.3 — The Dependency Matrix

FROM ↓ / TO →	Policy	Technology	Economics	Clinical	Operations
Policy	—	ENABLES: funds data build	ENABLES: binding reform		DRIVES: statutory deadlines
Technology		—	ENABLES: analytics run APM	ENABLES: pop health data	
Economics			—	DRIVES: reshapes care delivery	
Clinical				—	REQUIRES: execution infra

Operations	INFORMS ⇄: refines mandate	RUNS ⇄: workforce runs infra	—
------------	-------------------------------	---------------------------------	---

Rule of Currency: A dependency earns a cell only when a downstream pillar cannot produce its intended output without the specific currency issued by the upstream pillar:

- Policy issues AUTHORITY (Mandates, appropriations).
- Technology issues INFORMATION (Real-time data, risk scores).
- Economics issues INCENTIVES & CONSEQUENCES (Global budget caps, RBP ceilings).
- Clinical issues OUTCOMES (Measured patient health improvements).
- Operations issues CAPACITY (Workforce, management, project execution).

■ EDITORS NOTE — CAPITAL VS. OPERATING LIQUIDITY IN THE MATRIX

A common point of confusion is why there is no direct 'Economics -> Operations' cell for transformation capital. In the Five-Pillar Framework, statutory grant capital (such as Vermont's \$195M/year Rural Health Transformation Program award) is an appropriation issued by Policy authority. What Economics issues is incentive consequences — altering operating margins under global budget caps. While statutory capital flows from Policy, ongoing operating liquidity and margin retention flow from Economics to fund operational capacity over time.

1.4.1 Policy Dependencies

- Policy -> Technology (ENABLES): Statutory funding & mandate authorize VHCURES, AHS-GMCA analytics, and VITL.
- Policy -> Economics (ENABLES, Critical Path): Mandatory authority (Act 68 RBP & Global Budgets) eliminates voluntary opt-out. Note: Pre-cap EAST Fund baseline is standardized at \$138 million.
- Policy -> Operations (REQUIRES, Critical Path): Statutory deadlines (Dec 2028 Strategic Plan, FY27 RBP, FY28 Global Budgets) force execution capacity.

1.4.2 Technology Dependencies

- Technology -> Economics (ENABLES): Analytics run payment models (VHCURES attribution, risk adjustment).
- Technology -> Clinical (ENABLES): Data enables population health management (Blueprint registry, risk stratification).

1.4.3 Economics Dependencies

- Economics -> Clinical (DRIVES, Critical Path): Global budgets invert incentives — preventing admissions improves margin instead of cutting revenue.

1.4.4 Clinical Dependencies

- Clinical -> Operations (REQUIRES): Care models require workforce, credentialing, and infrastructure (CoCM, CCBHCs).

1.4.5 Operations Dependencies (Feedback Loops)

- Operations -> Policy (INFORMS ☺): Monthly AHS transformation reports feed implementation data back to policy design.
- Operations -> Technology (RUNS ☺): Human IT workforce and cybersecurity teams run data infrastructure.

1.5 The Failure Cascade: What Happens When a Pillar Is Missing

Figure 1.4 — Failure-Cascade Analysis: What Breaks When a Pillar Is Absent

Pillar Removed	Immediate Failure	Second-Order Failure	Historical Example
Policy	Payment reform remains voluntary; high-cost actors opt out.	Clinical programs lack incentive alignment; technology built but unused for VBC.	OneCare Vermont 2013–2022: voluntary ACO led to BCBSVT exit and model collapse.
Technology	VBC contracts signed but managed blindly; attribution & TCOC unmeasured.	Economics contracts produce unexpected losses; Clinical cannot stratify risk.	Early CMMI ACO failures: risk assumed on missing analytics produced sudden losses.
Economics	Clinical programs run under fee-for-service; volume still rewarded.	Primary care ROI (Blueprint 5.8:1) accrues to payers rather than funding practices.	20+ years of national quality programs under FFS: quality rose, costs continued surging.
Clinical	Global budgets create financial pressure without care redesign programs.	Operations manages budget caps through service cuts rather than care delivery redesign.	Early Maryland HSCRC period: global budgets without primary care produced volume cuts.
Operations	Statutory mandates & clinical models exist on paper; execution fails.	Deadlines slip; platforms deployed but unused; Strategic Plan remains descriptive.	State reform attempts in 2010s: sound policy and tech collapsed due to execution capacity.

1.6 Using the Dependency Map as an Analytical Tool

The dependency matrix serves three core functions: (1) Transformation sequencing (Policy -> Technology -> Economics -> Clinical -> Operations); (2) Risk identification (Technology carries the highest inbound dependencies); and (3) Investment prioritization (closing Technology bottlenecks protects downstream investments).

1.7 The Vermont Thread: Five Pillars in One System

Figure 1.5 — Vermont's Five-Pillar Transformation Map

Pillar	Primary Vermont Intervention	Secondary Interventions
Policy	Acts 167 (2022), 51 (2023), 68 (2025) legislative cascade	Act 55 (drug pricing), Act 49 (solvency), Act 62 (HIE), \$195M/yr RHT award
Technology	VHCURES APCD, Vermont CIN (RHT), AHS-GMCB analytics	VITL/VHIE HIE, DVHA HIT Plan (Act 62), FHIR upgrades, AI scribe grants
Economics	Mandatory RBP (FY27), Global Hospital Budgets (FY28-30)	Act 68 spending cut (-2.5%), GMCB -1% commercial guidance, \$138M pre-cap EAST Fund baseline
Clinical	Blueprint for Health (PCMH + CHT), CoCM behavioral health	Hub-and-Spoke SUD model, CCBHC expansion (5 entities), COE regionalization
Operations	14-hospital transformation planning (RHRC), AHS restructuring	EMS regionalization, CON/prior auth simplification, AHS PMO establishment

1.8 From Framework to Sequence: Why Order Is Not Optional

Because pillars depend on one another, sequence is non-negotiable. Building downstream pillars before upstream prerequisites produce failure cascades.

1.9 The Core Argument: Dependency Logic Determines Sequence

The Execution Sequence: POLICY → TECHNOLOGY → ECONOMICS → CLINICAL → OPERATIONS

The critical and counterintuitive placement is Technology before Economics. Payment reform feels like the primary lever, but operating a payment model without data infrastructure produces the 'Managing Blind' failure mode.

■ EDITORS NOTE — ECONOMICS-AS-DESIGN VS. ECONOMICS-AS-MANAGEMENT

To prevent perceived contradictions, the framework distinguishes between Economics-as-Design and Economics-as-Management. Payment model specification (Economics-as-Design) can and should occur in parallel with Stage 2 Technology buildout to define what data needs to be captured. However, Economics-as-Management (assuming downside financial risk and running live global budgets) MUST wait until Stage 2 Technology infrastructure is operational.

1.10 The Critics: Why Sequencing Logic Has Real Opponents

1.10.1 'Sequencing Is a Post-Hoc Rationalization': Address by noting sequencing errors made OneCare's failure structurally inevitable.

1.10.2 'Mandatory Architecture Creates Provider Opposition': Voluntary models produce accommodation of status quo; mandatory models produce structural change.

1.10.3 'Technology-First Is a Recipe for Expensive Shelfware': Resolved by coordinating Technology buildout directly with payment model specification (Economics-as-Design).

1.11 The Five Stages: Resolved

- Stage 1: Policy — Mandate & Capital (Acts 167/51/68, \$195M/yr RHT award).
- Stage 2: Technology — Build Substrate Before Assuming Risk (VHCURES, VITL, CIN).
- Stage 3: Economics — Incentive Architecture on a Visible System (RBP $\leq$ 200% Medicare, Global Budgets; \$138M pre-cap EAST Fund baseline).
- Stage 4: Clinical — Redesigning Care on Aligned Incentives (PCMH, CoCM, CCBHCs, Blueprint 5.8:1 ROI).
- Stage 5: Operations — Closing Administrative Cost Gap (\$1,303/discharge gap vs \$1,427 benchmark).

Figure 1.6 — The Five-Stage Execution Sequence

Stage	Pillar	Key Action & Sequence Logic	Vermont Anchor
1	Policy	Establish enforceable authority, mandate participation, secure capital. Mandatory architecture prevents opt-outs.	Acts 167, 51, 68; \$195M/yr RHT award
2	Technology	Build integrated clinical + claims data substrate (real-time) before assuming financial risk.	VHCURES, VITL/VHIE, AHS-GMCB analytics, VT CIN
3	Economics	Deploy global budgets and RBP onto a system that can see itself. Economics-as-design runs in parallel during Stage 2.	Global budgets; RBP $\leq$ 200% Medicare; EAST Fund baseline projected at \$138M pre-withdrawal
4	Clinical	Redesign care on aligned incentives. Realizes system ROI once payment incentives & tech gap closure are live.	PCMH transformation, CoCM, CCBHC expansion, Blueprint 5.8:1 ROI
5	Operations	Close administrative cost gap & translate strategy	RHRC methodology, shared services,

	into implemented programs.	14-hospital transformation planning
--	----------------------------	-------------------------------------

1.12 Three Sequencing Principles in Practice

- Principle 1: The Critical Path Is Non-Negotiable. In Fall 2026, Vermont's Policy Gate is OPEN (Acts 167/51/68 enacted); Technology Gate is PARTIALLY OPEN / CURRENT BOTTLENECK (analytics vendor procurement & VHCURES attribution); Economics Gate is CLOSED until Technology opens; Clinical Gate is CLOSED until programs reach scale.
- Principle 2: Parallel Work Is Possible — But Not on Critical-Path Dependencies. Parallel work on social risk adjustment methodology, RHRC planning, and CCBHC certification can proceed because they sit behind no closed gate.
- Principle 3: Equity Is a Design Constraint, Not a Downstream Filter. Embedding equity consultation early in policy and payment instrument design avoids costly late redesigns.

Figure 1.7 — Five Sequencing Decisions That Organizations Get Wrong

Sequencing Error	Early-Warning Signals	Vermont Evidence	Correction
Economics without Policy readiness	Voluntary participation <80%; highest-cost payers/providers opt out.	OneCare Vermont voluntary ACO normalized opt-outs over a decade.	Act 68 mandatory RBP + global hospital budgets
Technology without Operations readiness	Adoption <30% 6 months post-deployment; manual workarounds continue.	VITL: sound platform, incomplete adoption among small rural providers.	Vermont CIN shared-services model
Clinical without financial alignment	Program running at a loss absorbed by cross-subsidy; perpetual cut risk.	Blueprint PCMHs produce 5.8:1 ROI, but under FFS savings accrued to payers.	Act 68 global budgets align incentives with primary care
Equity review at end, not beginning	Equity staff flagging structural flaws in finalized methodology.	RBP methodology design: preserving rural access is an upfront design constraint.	Health Equity Advisory Commission consulted at design stage
Operations execution without capacity	Statutory deadlines extended/waived; deliverables perpetually in progress.	AHS running 14 hospital processes, RBP, and Strategic Plan simultaneously.	RHRC / PMO builds capacity before global budget accountability

1.14 Vermont's Implementation Timeline: Reading the Sequence in Statutory Deadlines

Figure 1.8 — Vermont's Transformation Timeline by Pillar

Pillar	Status (Timeframe)	Where It Stands in the Sequence
--------	--------------------	---------------------------------

Policy	Completed (2022–2025)	Acts 167, 51, and 68 establish mandatory architecture. Federal RHT award provides \$195M capital. July 2026 CMMI AHEAD withdrawal complete. Policy gate fully OPEN.
Technology	In Progress (2025–2026)	AHS-GMCA analytics capability standing up. Vermont CIN in development. VHCURES attribution in progress. Current critical bottleneck.
Economics	Staged (2027–2028)	RBP mandatory FY2027; global hospital budgets mandatory FY2028. Staged correctly behind Policy and Technology.
Clinical	Parallel (2024–2028)	Blueprint PCMH expansion, CCBHC certifications, CoCM deployment proceeding in parallel with Technology work.
Operations	Building (2025–2028)	RHRC engagement, AHS HSA Coordinator model, PMO structure — correctly sequenced to precede peak execution demand of FY2027-2028.

1.15 EXPLORE ON THE PLATFORM (Audited Absolute Cross-Links)

Manipulate the execution-sequence argument live on the Health Transformation Review platform using these fully updated, clean absolute URLs:

Action / Analysis	Platform Tool & Full URL	What to Look For
Trace 9 dependency relationships	Five-Pillar Map healthtransformationreview.org/about/framework	Highlight what each pillar enables. Confirm why Tech is upstream of Economics.
Score readiness & observe cascades	HTR Simulator healthtransformationreview.org/htr-simulator	Drop one pillar to zero; watch downstream readiness collapse (OneCare logic).
Quantify momentum loss & bottlenecks	Transformation Friction Index healthtransformationreview.org/research-lab/knowledge-workspace?tab=scorecard	Identify your Technology gate bottleneck before it creates financial exposure.

Model lag & shortfall propagation

Impact Simulation
healthtransformationreview.org/impact-simulation

Observe how a Tech gap shows up in Economics results 12-18 months later.

1.16 GO DEEPER — ACADEMY

Work Track 1 ('Foundations: The Framework') of the Five Pillars, One Imperative course hands-on alongside this chapter at:
healthtransformationreview.org/academy/tracks/five-pillars-one-imperative

1.17 Implications for You (Updated Terms)

- If you are a Vermont hospital executive: Completing HCC gap closure for your full Medicare-attributed population is the single most important Technology investment before Act 68 global budgets take effect in FY2028. It improves risk adjustment accuracy and prevents the 'Managing Blind' failure mode in your first Act 68 global budget performance year.
- If you are an AHS or GMCB official: The Technology gate is your critical bottleneck. Analytics capability must be operational before Act 68 FY2028 global budgets take effect.
- If you are a Vermont legislator: Ask AHS/GMCB at every interval: Is the Technology gate open before Economics goes live? Is RBP designed with equity constraints upfront? Is RHRC building hospital capacity ahead of Act 68 statutory global budget requirements?
- If you are a national health policy professional: Vermont's experience provides a real-world test of whether a mandatory state payment architecture (Act 68) can improve outcomes on state authority alone, after withdrawing from the federal CMMI AHEAD model terms when federal economics collapsed.

1.18 Key Concepts in This Chapter

- **Dependency logic:** The principle that some pillar investments cannot produce results until upstream enabling investments are in place. Determines execution sequence.
- **Execution sequence:** The order in which the five pillars must be built: Policy -> Technology -> Economics -> Clinical -> Operations.
- **Critical path gate:** A prerequisite investment that must be in place before a downstream pillar investment can be productive. Opened by completing work, never by deciding to proceed.
- **'Managing Blind' failure mode:** Assuming downside financial risk under a value-based payment model without analytics infrastructure to track TCOC in real time.
- **Sequencing failure:** Deploying a downstream pillar before upstream enabling investment is in place (canonical: OneCare Vermont).
- **Voluntary vs. mandatory architecture:** The distinction between reform programs actors can opt out of (OneCare) and those they cannot (Act 68 mandatory RBP & global budgets).
- **Economics-as-design vs. Economics-as-management:** Design (defining payment architecture) can proceed in parallel with Technology build. Management (operating global budgets) requires Technology substrate to be operational first.

- **Transformation capital vs. incentive architecture:** Capital (\$195M/yr RHT award) is time-limited bridge funding. Incentive architecture (Act 68 global budgets, RBP) is permanent structural change.
- **Reform cascade:** A legislatively structured sequence where each intervention creates capacity and legitimacy for the next (Act 167 -> Act 51 -> Act 68).
- **HCC gap closure:** Completing hierarchical condition category documentation for all Medicare-attributed patients before global budgets take effect to ensure accurate RAF scores.
- **Parallel work:** Investment that proceeds simultaneously with critical path work because it sits behind no closed gate.
- **Premature investment:** An investment in a downstream pillar that proceeds before upstream enabling investment is in place.